

VANGUARD UNIVERSITY MEDICAL CENTER

1800 BioInnovate Dr, Philadelphia, PA 19103 | (215)555-6601 | www.vanguardumc.org

DISCHARGE SUMMARY

Patient Name: Abimbola Adewale	DOB: July 11, 1987 (Age 38)
MRN: MRN-3889596	Gender: Male
Admission Date: September 19, 2025	Discharge Date: September 27, 2025 (8 days)
Attending Physician: Dr. Dominique Lefevre, MD — Critical Care / ICU	Insurance: Medicare #MCR-A1-11111
ICU Admission: YES — MICU/SICU	ICU Days: 4 of 8 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Septic Shock — Gram-negative Bacteremia	R65.21	Acute / Critical
Type 2 Diabetes — Uncontrolled (HbA1c 11.2%)	E11.65	Chronic / Complicating
Metastatic Non-Small Cell Lung Cancer	C78.9	Chronic / Complicating

HOSPITAL COURSE

Abimbola, a 38-year-old male with a history of Type 2 Diabetes — Uncontrolled (HbA1c 11.2%) and Metastatic Non-Small Cell Lung Cancer, was admitted to the Medical Intensive Care Unit (MICU) following rapid deterioration from septic shock secondary to gram-negative bacteremia. Blood cultures on admission grew E. coli. The patient required vasopressor support (norepinephrine and vasopressin) for 72 hours. Broad-spectrum antibiotics were initiated (meropenem + vancomycin) and de-escalated to ceftriaxone upon sensitivities. Acute kidney injury with creatinine peaking at 6.2 mg/dL required continuous renal replacement therapy (CRRT) for 4 days. The patient was successfully weaned from vasopressors by day 5 and CRRT was discontinued on day 7 after renal recovery. ICU length of stay was 9 days. Transferred to step-down unit and discharged on day 14 with close nephrology follow-up.

VITALS AT DISCHARGE

■ NOTE: Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
117/67 mmHg	112 bpm	94% (on 2L O2)	100.0°F	22/min	108 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Propofol	Per protocol	Continuous IV sedation	Discontinued prior to discharge
Levetiracetam	1500 mg	Twice daily	Ongoing — neurology follow-up
Piperacillin-Tazobactam	4.5g IV	Every 6 hours	7-day course
Vancomycin	1.5g IV	Every 12 hours (AUC-guided)	14-day course
Metronidazole	500 mg	Every 8 hours	7-day course
Vasopressin	0.03 U/min	Continuous IV infusion	Weaned and discontinued in hospital

FOLLOW-UP PLAN

URGENT: Critical Care / ICU: Dr. Dominique Lefevre, MD — within 72 hours of discharge (September 30, 2025).
Primary Care: Within 1 week (October 04, 2025). Full labs including BMP, CBC, LFTs.
Home health nursing: Daily vitals and wound check for first 7 days post-discharge.
Inpatient rehabilitation / LTACH: Transfer arranged — starts October 11, 2025.

■ **HIGH-RISK DISCHARGE — Patient requires close monitoring. Re-admission risk: HIGH.**

Attending Physician Signature: _____

Dr. Dominique Lefevre, MD — Critical Care / ICU | License #: PA-MD-21345 | Date: September 27, 2025

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